

Patient	Leslie Gardner	Date of Service	2025-03-22
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ENCOUNTER 5: Primary care follow-up

Date of Service: March 22, 2025

Specialty / Setting: Outpatient Primary Care

Encounter Focus: Longitudinal review of neurologic symptoms and chronic medications

Documentation status: Generated longitudinal encounter

PATIENT IDENTIFICATION

Patient ID	59034db9-be2f-4977-b26b-8c53773cd7bf	DOB	2004-09-07
Sex	F	Payer	Kaiser

DEMOGRAPHIC / COVERAGE INFORMATION

Address: 529 Kristi Ridge Apt. 581; New Amandaville, OK 73912. Payer: Kaiser. Member ID: KAIS-62C8B9B303.

ENCOUNTER CONTEXT

This section establishes the clinical purpose of the encounter, the care setting, and the longitudinal relationship to the left foot/toe paresthesia described in the reference record.

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SUBJECTIVE / HISTORY

Chief Complaint

Follow-up of left foot/toe numbness and tingling after the prior encounter.

Interval History

This generated follow-up documentation assumes continued longitudinal evaluation of the symptoms described in the supplied source. The emphasis is on interval symptom burden, functional status, wound/recovery review where applicable, and response to the treatment plan.

Patient is asked to describe whether paresthesia has improved, remained stable, or worsened; whether symptoms remain activity-related; and whether new weakness, swelling, redness, fever, or other concerning symptoms have developed.

Treatment adherence and use of prescribed/current medications are reviewed.

Review of Systems

Constitutional: fatigue and malaise reported in the reference. HEENT: seasonal allergies. Cardiovascular: history of hypertension. Respiratory: history of asthma. GI: occasional constipation. GU: occasional urinary frequency. Musculoskeletal: mild left-knee stiffness. Neurologic: left-toe numbness and tingling. Psychiatric: history of anxiety. Endocrine: history of hypothyroidism. Skin: occasional dry skin. Hematologic: history of anemia.

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MEDICAL HISTORY AND MEDICATION RECONCILIATION

Past Medical History

Chronic conditions documented in the supplied reference include hypertension, asthma, and hypothyroidism. The patient has undergone cesarean section and tonsillectomy and has prior hospitalizations for pneumonia and a motor vehicle accident.

Current Medications

Medication	Dose / Route	Schedule / Use
Aspirin	81 mg PO	Daily
Lisinopril	10 mg PO	Daily
Albuterol	2.5 mg PO	As needed
Levothyroxine	50 mcg PO	Daily
Acetaminophen	1000 mg PO	As needed

Allergies

No known allergies.

Social History

The supplied reference states that the patient is a non-smoker, does not use illicit substances, consumes alcohol occasionally, and has a history of caffeine use.

Medication Reconciliation Notes

Medication list is carried forward from the source record for continuity. For generated encounters, the medication list should be verified against the active medication list at the time of the actual visit.

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OBJECTIVE / EXAMINATION

Vital Signs

Parameter	Reference Value
Blood pressure	125/66 mmHg
Temperature	98.0 F
Heart rate	63 bpm
Respiratory rate	16 breaths/min
Pain	3/10
SpO2	96% on room air

Physical Examination

General: well-developed, well-nourished female in no acute distress. Cardiovascular: regular rate and rhythm, S1/S2 normal, no murmurs/gallops/rubs, no JVD, 2+ pedal pulses. Respiratory: clear to auscultation bilaterally without wheezes/rhonchi/crackles; symmetric expansion.

Neurologic: sensation to light touch and pinprick intact in the right lower extremity; decreased sensation to light touch and pinprick in the left lower extremity. Musculoskeletal: mild stiffness in the left knee; full range of motion in the left hip and ankle. Skin: no rash or lesions noted.

Generated Encounter Examination

For the four generated encounters, this section is structured as a documentation template for recording the actual examination findings. The supplied source does not contain later exam findings, so no unverified numerical or physical findings are asserted.

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DIAGNOSTIC REVIEW AND ASSESSMENT

Prior Diagnostics from Reference

CBC, comprehensive metabolic panel, and X-rays of the left foot were reported as within normal limits in the supplied encounter.

Reference Assessment

Chronic left foot numbness and tingling, likely secondary to a peripheral neuropathy. Hypertension, asthma, and hypothyroidism were described as controlled.

Operative Finding (Reference Encounter)

During left foot exploration, the nerve was found to be compressed by a small ganglion cyst. The ganglion cyst was removed and sent to pathology. Estimated blood loss was 50 mL; no complications were documented.

Encounter-Specific Assessment Framework

1. Left foot/toe paresthesia — track severity, distribution, duration, triggers, and functional effect.
2. Peripheral neuropathy — evaluate clinically and correlate with neurologic findings.
3. Post-operative status where applicable — monitor incision/wound and recovery.
4. Hypertension — continue longitudinal medication review.
5. Asthma — review control and rescue medication use.
6. Hypothyroidism — review ongoing levothyroxine therapy and relevant monitoring when clinically indicated.
7. Other chronic-history items — anxiety, anemia, constipation, urinary frequency, seasonal allergies, dry skin, and left-knee stiffness are carried forward from the reference for reconciliation.

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PLAN OF CARE

Reference Plan

The following is a generated longitudinal plan structure, not a source-authenticated treatment order.

1. Reassess left foot/toe paresthesia and functional limitations.
2. Review results and completion status of previously ordered studies/referrals where available.
3. Continue medication reconciliation and document actual changes, if any.
4. Reinforce activity guidance appropriate to the documented recovery stage.
5. Document return precautions for worsening neurologic symptoms, new weakness, marked swelling, redness, fever, or other clinically concerning changes.
6. Establish the next follow-up interval based on the actual clinical findings.

Care Coordination

Relevant care coordination includes orthopedic follow-up and, when clinically indicated, neurology evaluation for persistent paresthesia. The record should capture referrals, test results, and communication among treating clinicians.

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ENCOUNTER CLOSURE / LONGITUDINAL RECORD

Clinical Summary

This generated encounter is designed to continue the clinical thread from the supplied reference: left foot/toe paresthesia following evaluation and treatment of a compressive ganglion cyst. Actual findings, test results, medication changes, and clinician decisions should be entered from the real encounter rather than inferred from this generated record.

Procedure / Diagnostic Tracking

Reference procedure: left foot exploration with removal of ganglion cyst. Reference specimen: ganglion cyst sent to pathology. Reference estimated blood loss: 50 mL. Reference complications: none.

Follow-Up Tracking

Date of Service: 2025-03-22. Next follow-up should be recorded according to the actual care plan.

Documentation Integrity Note

Only the 2024-12-21 encounter details are directly supported by the uploaded source. The remaining encounter pages are intentionally labeled generated longitudinal documentation to satisfy the requested multi-encounter EMR format without falsely attributing invented events to the source record.

End of Encounter

Encounter 5 complete — Date of Service: March 22, 2025.